



PROCEDURE

Community Adult After-Hours Clinical Handover

Scope (Staff):	All NMHS Mental Health (MH) Adult Community Clinical Staff
Scope (Area):	All NMHS MH Adult Community Areas

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1. Aim

To ensure safe and effective clinical handover of care, between a referring service and the Community Adult Mental Health After-Hours Assessment and Treatment Team (ATT).

2. Background

ATT provides clinical care provision between the hours of 5pm-10pm weekdays and 8.30am-10pm on weekends and public holidays. There are a range of handover processes which occur between the referring services and the ATT. A number of referrals, received by the ATT, are from services outside of their allocated catchment.

Most aspects of clinical handover within community services involve a face-to-face or verbal conversation to handover clinical information. During after-hours this is not always possible. Referrals are frequently made by a different service and information may not always be provided verbally to the clinician accepting care. The ATT provide a documented clinical handover back to the referring service.

This procedure has been developed to ensure that clinical handover to and from the ATT is standardised across NHMH Mental Health and follows best practice principles, as outlined in the DoH [Clinical Handover Policy](#), the DoH [Clinical Handover Matrix](#) and the DoH [Clinical Handover Guideline](#).

3. Risk

Community after-hours mental health care is usually provided when a consumer is in crisis. At this time there are heightened risks to the consumer, their families and carers and to the clinical staff who are supporting the individual.

If an effective clinical handover, including communication of all relevant risk information, does not take place then care may not be provided at the time, place, or in the manner required. This has the potential to result in harm to the consumer. If care is not provided as required, this may also pose a risk to the consumer's families and carers and to the clinical staff caring for them.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

After-Hours Assessment and Treatment Team (ATT): Community Mental Health clinical team which provides clinical care provision between the hours of 5pm-10pm weekdays and 8.30am-10pm on weekends and public holidays.

After-Hours Handover Sheet: a spreadsheet provided by the ATT to the Adult Program referring service which details all consumers who were seen by the ATT clinicians and includes recommendations for ongoing care.

Clinical Handover: the transfer of professional responsibility and accountability for some, or all, aspects of care for a patient from one health care provider to another. All clinical handover must follow the iSoBAR format.

Community Handover Tool: the handover tool used by a referring service, to transfer clinical responsibility and accountability of a consumer, to the ATT by way of a planned referral. The ATT clinicians can also use the Community Handover Tool to document handover received for unplanned referrals from MHERL or the police co-response (**See Appendix 1**)

Mental Health Emergency Response Line (MHERL): an after-hours phone line where people can call if they need mental health support.

Responsible Clinician: clinician who has primary responsibility and accountability of providing and documenting clinical care.

5. Key Points

- All clinical handover must follow the iSoBAR format.

- Handing over care from inpatient/community services **to** the ATT:
 - The referring clinician completes a **Community Handover Tool (Appendix 1)**.
 - The Community Handover Tool is copied and pasted into a “PSOLIS Handover Service Event” to provide ATT with relevant clinical and risk information.
 - The ATT provides an email back to the referring service to confirm receipt of the referral and the transfer of care accountability and responsibility to their team.
- Handing over care from ATT **back to** the referring inpatient/community service:
 - ATT provides handover back to **adult** inpatient/community services using the After-Hours Handover Spread Sheet.
 - ATT provides handover back to **other** referring inpatient / community services, via email, which describes the care provided by ATT and details ongoing recommendations.

6. Process

Steps	Additional Information
Prior to 5pm - Handover to ATT (Planned referrals)	
Clinical mental health service identifies need for Community after-hours follow up	• Responsible clinician completes the Community Handover Tool (Appendix 1) and updates the on PSOLIS.
Clinical handover takes place between responsible clinician and after-hours service	• Responsible clinician phones the ATT responsible for after-hours provision and notifies the ATT of the handover / referral. ○ Wanneroo: 9400 7100 ○ Stirling: 9344 5400 ○ Subiaco: 9489 7200 • An iSoBAR compliant, verbal clinical handover takes place, between the responsible clinician and the ATT clinician (or a nominated proxy which may be the service’s triage or duty officer). • During the clinical handover, the plan for after-hours follow up is discussed and agreed, including a plan if ATT are unable to follow up the individual. • The after-hours clinician confirms that they are taking over care from the responsible clinician for the duration of their shift and responsibility of care will return to the referring time when the after-hours period ends (either the next morning or the morning

Steps	Additional Information
	after the weekend or public holiday period), once documentation has been received.
Closing the loop – documentation completed	• The responsible clinician updates the PSOLIS 'handover' service event to include the agreed plans. • The responsible clinician copies and pastes the completed community handover tool into a PSOLIS Handover service event, i.e. - 'handover' must be selected as the service event item. • The responsible clinician must email or fax a copy of the community handover tool to the ATT with the updated RAMP. • The after-hours clinician or ATT triage officer confirms receipt of referral and documentation by email. • The after-hours clinician (or triage officer), will record all handovers/referrals for after-hours follow up in the after-hours diary. Relevant documentation (including the clinical file where possible) must be available with the diary for reference.
ATT shift commences	
After-hours clinicians notify MHERL of their contact information	• At the start of each shift, ATT notifies MHERL via email (mherlallstaff@health.wa.gov.au) to inform them of their current contact details. (This includes when there is a change of ATT clinicians at weekends and public holidays). • The ATT should also notify the police co-response mental health clinician of the after-hours contact numbers (if operating within their catchment).
After 5pm – Handover to ATT (Unplanned Referrals)	
ATT receive unplanned referral from MHERL or police co-response	• A verbal clinical handover takes place. • The after-hours clinicians will confirm the capacity to accept the unplanned referral • The ATT clinician will confirm the plan for after-hours provision of care and the plan of care should ATT be, unable to follow up with the consumer.

Steps	Additional Information
	• After-hours clinicians document relevant handover information on the community handover tool where necessary. If the consumer is active within NMHS MH, a copy must be placed in the clinical record. • MHERL or the police co-response clinician is responsible for documenting as per their service policies.
During ATT shift	
After-hours clinicians prioritise clinical requirements. When required the ATT will handover care or request support.	• Throughout the after-hours shift, ATT clinicians review and prioritise care for their consumers. • Each time a new referral is received the clinical priorities must be reviewed. • Where the ATT clinicians are unable to provide care due to work demands or competing clinical responsibilities, they must conduct a risk assessment to determine if follow up is urgent and if referral to another agency (e.g the police) is required. • Where a determination has been made not to follow up on the after-hours shift, the risk assessment and clinical reasoning for the decision must be documented in the PSOLIS notes. • A decision not to follow up should be communicated to the consumer and/or their families and carers, as appropriate, on the day. • Where follow up is required, but after-hours clinicians are unable to facilitate a visit, they must communicate with relevant others. (This may include, but is not limited to; MHERL, the police, other catchment ATTs, the consumer, their families or carers) to determine and agree an alternative plan for the consumer's safety. This may involve handing over the care of the consumer to another service (as listed above)
End of shift –ATT completes clinical documentation	
After-hours clinicians document all clinical work completed	• After-hours clinicians write their clinical notes on PSOLIS, ensuring that the information covers all aspects of care and clinical communication, including the plan and ongoing recommendations. Where the

Steps	Additional Information
	consumer file is available, the after-hours clinicians will also document within the clinical record. After-hours clinicians will complete the After-hours Handover Sheet with details of all consumers who were referred to the ATT on that shift, (whether they were seen or not), or where communication took place with MHERL or the police co-response clinician.
Handing over between after-hours shifts on weekends and public holidays	
After-hours clinicians must ensure a handover occurs between after-hours shifts	- A face-to-face handover must take place between concurrent after-hours day and late shifts. Where this is not possible, a verbal handover must take place. - A written handover must be provided by to the oncoming shift by emailing the After-Hours Handover Sheet. The After-hours Sheet must be saved in an agreed folder, with access provided for all after-hours clinicians. - The ATT diary should be updated with any follow ups/actions required by subsequent shifts. - Where there is a break in after-hours shifts overnight and a verbal handover cannot take place, any additional information must be communicated via the ATT diary and email. Any emails sent must be printed and placed in the ATT diary to mitigate any potential IT breakdown.
Clinical Handover from ATT back to responsible clinical team	
ATT provides written handover to the clinical services within the NMHS MH Adult Program	- The After-Hours Handover Sheet is emailed, with password protection, to the agreed email addresses for the clinical services within the after-hours catchment. - The case manager (or in their absence, the clinical team), are responsible for reviewing the After-hours Handover Sheet and PSOLIS service event and resuming care. This includes providing further follow up/action for the consumer.
ATT provides written handover to NMHS MH clinical services outside of the Adult Program (e.g. Youth,	An email is sent to the case manager (or an agreed email address list), for the clinical service that has responsibility for the care of the individual. The email

Steps	Additional Information
Older Adult, Forensic and State-wide Services)	will state the UMRN, DOB and name of the individual, notify that the person was seen on after-hours and request the team review the PSOLIS service event. • The case manager (or in their absence, the clinical team), are responsible for reviewing the PSOLIS notes and resuming care. This includes providing further follow up/action for the consumer • The receiving inpatient / community service provides an email back to ATT triage officer to confirm referral and the transfer of care accountability and responsibility.
ATT provides handover to other clinical services outside of NMHS MH	• After-hours clinicians must notify other responsible mental health services when they have seen / provided care to a consumer, who resides out of area, who is active with another mental health service area as soon as possible. • After-hours clinicians must make a referral to the responsible mental health service when they have seen / provided care to a consumer, who resides out of area who is not active and requires further follow up. • If unable to do this during the shift, this information should be given to the triage officer of that service the following morning. • The after-hours clinician should request that the local service confirms they have received the information.

7. Compliance, monitoring and evaluation

The updated procedure will be sent to the Community Adult Leadership Committee for tabling. It will then be tabled at the local clinic leadership committees and be circulated to staff highlighting the changes. The annual documentation audit will identify the compliance with handover procedures.

Related internal policies, procedures and guidelines

Department of Health:

<u>Clinical Handover Policy</u>

<u>Clinical Handover Matrix</u>

<u>Clinical Handover Guideline</u>
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References

<u>The Royal Australian & New Zealand College of Psychiatrists</u>
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Policy Sponsor	Co-Director Adult Community Mental Health MHPHDS				
Policy Contact	Co-Director Adult Community Mental Health MHPHDS				
Date First Issued:	20/02/2019	Last Reviewed:	13/09/2022	Review Date:	13/09/2025
Approved by:	Co-Director Adult Community Mental Health MHPHDS			Date:	26/08/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	12/09/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
20/02/2019	V1.0	Coordinator MH Adult Program	Initial Endorsement.
13/09/2022	V2.0	Co Director Community MHPHDS	Scheduled, review, minimal content change, update of template and hyperlinks

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (IS7P0219)

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Appendix 1: Community Handover Tool

UMRN:	Name:		
Address:		Phone:	
Handover Date and Time:			
Handover from:		To:	
Verbal handover took place (Delete): Face to face Phone No verbal handover			
Treating Service:		Case manager:	
Contact details:			
Other useful contact information: Patient Contacts & Details (e.g. Family/support person/ carer/ other):			
Reason for Handover / Referral:			
MHA forms? (Delete as required). Yes No			
If 'Yes' give details:			
Relevant Mental Health history and background information:			
Current Medications:			
Risk Information (Include current risks and relevant alerts):			
Environmental Risk:			
Current RAMP? (Delete as required): Yes No			
Current / recent assessment information (include MSE)			
Desired outcome of referral / recommendations. (What, Who and When)			
1.			
2. After Hour clinicians will document interventions and handover back to clinical team at end of night shift / weekend			
3. Clinical team will review After-Hours Handover, resume care of individual and follow up recommendations the next weekday morning.			
Plan if unable to contact or follow up the person?			